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Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

PRESIDENTIAL EBOLA TASK FORCE 17

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°152 /MVE-BDBV/13/10/2026

Report date: October 13, 2026 Publication date: October 14, 2026

HIGHLIGHTS (last 24 hours)

- Sixty-eight (68) new confirmed cases of Ebola Virus Disease (EVD), including 20 community deaths, have been reported. These cases originated from the provinces of Ituri (41 cases), North Kivi (25 cases), and Haut-Uele (2). In addition, 11 deaths were recorded in the CTEs.
- A total of 394 new samples were analyzed; 68 of these samples tested positive for the virus. Bundibugyo, representing a 17.3% positivity rate

ACCUMULATIVE CAS	ACCUMULATIVE DEATH CONFIRMED	LETHALITY	PATIENTS IN ISOLATION/CTE	ACCUMULATIVE HEALED	TRACKING RATES CONTACTS (Of the Day)
9212	4 441	48,2 %	903	2 448	81,8%

Affected provinces and affected health zones

7 Provinces affected

Uele, Ituri, North Kivu, South Kivu, Tshopo, Lower Uele and Sud Ubangi

65 Health Zones affected out of 167 (38,9%)

Summary of key points from the last 24 hours

In the last 24 hours, **68 new confirmed cases** of Ebola virus disease (EVD) have been reported in three provinces: **Ituri (41 cases), North Kivu (25 cases) and Haut-Uélé (2 cases)**

In addition, **11** deaths were recorded in the CTEs (6 in Ituri and 5 in North Kivu, This brings the total number of confirmed deaths for the period in question to 31.

Furthermore, **30** patients **(13 in Ituri, 14 in North Kivu, 2 in Haut-Uélé and 1 in the** Tshopo) were declared cured after obtaining two negative results in the follow-up test for Ebola Virus Disease.

Since the start of the epidemic, the DRC has recorded **9,212 confirmed cases and 4,441 deaths,** representing a case fatality rate of **48.2** %. Ituri remains the epicenter of the epidemic, accounting for **73.5% of cumulative confirmed cases and 60.3% of new confirmed cases.** reported in the last 24 hours.

65 out of 167 health zones (38.9%), spread across seven provinces, have been affected since the start of the epidemic.

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Table 1. Distribution of confirmed cases and deaths by affected province, as of October 13, 2026.

Province	New cases confirmed (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	41	6775	3 131	46,2%	28/36 (77,8 %)
North Kivu	25	1963	1 117	56,9%	17/34 (50,0 %)
Haut-Uélé	2	393	165	42,0%	7/13 (53,8 %)
Boss	0	63	22	34,9%	7/23 (30,4 %)
South Kivu	0	3	1	33,3%	1/34 (2,9 %)
Bas-Uélé	0	13	4	30,8%	4/11 (36,4%)
Sud Ubangi	0	2		50,0%	1/16 (6,3%)
Total	**68**	**9 212**	**1 4 441**	**48,2%**	**65/167 (38,9%)**

Table 2. Distribution of confirmed cases and deaths by province and zone as of October 13, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cas (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	**6775**	**3131**	**46,2%**	**41**	**8**	**6**	**14**
Give	11	1	9,1%				0
Understand	9	6	66,7%				0
Ariwara	8	2	25,0%				0
Aungba	12	5	41,7%				0
Bamboo	216	48	22,2%	2	0	0	0
have	2	2	100,0%				0
Bunia	1859	616	33,1%	11	1	1	2
Ladies	40	9	22,5%				0
Drodro	15	7	46,7%				0
Fireworks	84	31	36,9%				0
Gethy	11	7	63,6%				0
Flounder	2	0	0,0%				0
Kilo	38	12	31,6%				0
Team	185	111	60,0%	3	1	1	2
Liter	305	155	50,8%	1	0	0	0
Logo	14	8	57,1%				0
Fight	36	9	25,0%				0
distributed	4	1	25,0%				0
Washing	60	22	36,7%	15	4	0	4
Darkness	269	139	51,7%				0
Mangala	372	162	43,5%	2	1	0	1
Mongbwalu	713	342	48,0%	1	0	0	0
Nanny	329	186	56,5%	2	1	2	3
Nizi	819	293	35,8%	3	0	2	2
Legumes	133	39	29,3%				0
Jungle	19	5	26,3%	1	0	0	0

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Rwampara	1134	395	34,8%				0
Tchomia	76	25	32,9%				0
A valves	THAT	493	THAT				0
North Kivu	**1963**	**1117**	**56,9%**	**25**	**10**	**5**	**15**
Me	482	322	66,8%	11	5	1	6
Farm	23	7	30,4%				0
Butembo	300	185	61,7%	3	0	0	0
Ten	1	0	0,0%				0
Kalunguta	73	28	38,4%	1	1	0	1
Cut	632	385	60,9%	2	1	1	2
Source	3	2	66,7%				0
Kyondo	40	21	52,5%	2	1	0	1
Lubero	29	13	44,8%	1			0
Be anxious.	66	26	39,4%	2	2	2	4
Alimbong	5	3	60,0%				0
Manguredjipa	15	5	33,3%				0
Masereka	37	12	32,4%				0
The museums	147	69	46,9%				0
Mutwanga	23	5	21,7%				0
Oicha	47	20	42,6%	2	0	0	0
Vuh	40	14	35,0%	1		1	1
Upper Uele	**393**	**165**	**42,0%**	**2**	**2**	**0**	**2**
Government Mangbetu	44	19	43,2%				0
Dumb	1	1	100,0%				0
Gombari	3	1	33,3%				0
Mathematics	104	46	44,2%				0
Lining	101	50	49,5%	1	1		1
Beans	5	2	40,0%				0
Ordinary	135	46	34,1%	1	1		1
Boss	**63**	**22**	**34,9%**	0	0	0	0
Bafwasende	10	1	10,0%				0
Kabondo	7	1	14,3%				0
Lubunga	1	0	0,0%				0
Maquis - Kisangani	32	12	37,5%				0
Mangobo	10	7	70,0%				0
Boss	2	0	0,0%				0
Wanie -Rukula	1	1	100,0%				0
South Kivu	**3**	**1**	**33,3%**	0	0	0	0
Miti -Murhesa	3	1	33,3%				0
Bas Uélé	**13**	**4**	**30,8%**	0	0	0	0
Blind	1	1	100,0%				0
Viadana	6	1	16,7%				0
Claim	1	0	0,0%				0
Walk	5	2	40,0%				0
Sud Ubangi	**2**	**1**	**50,0%**	0	0	0	0
Hair	2	1	50,0%				0
Total	**9212**	**4441**	**48,2%**	**68**	**20**	**11**	**31**

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*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA : Not Applicable.

Table 3. Status of alerts notified as of October 13, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	107	10	117	18	10	46	0	28	6
Bas Uélé	117	3	120	7	3	110	0	10	0
Ituri	1 462	86	1 548	163	86	1 299	0	242	143
North Kivu	1 092	36	1 128	180	35	912	1	215	164
Sud Ubangi	80	0	80	0	0	63	0	0	0
Boss	145	2	147	9	2	136	0	11	4
Total General	**3 003**	**137**	**3 140**	**377**	**136**	**2 566**	**1**	**506**	**317**

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

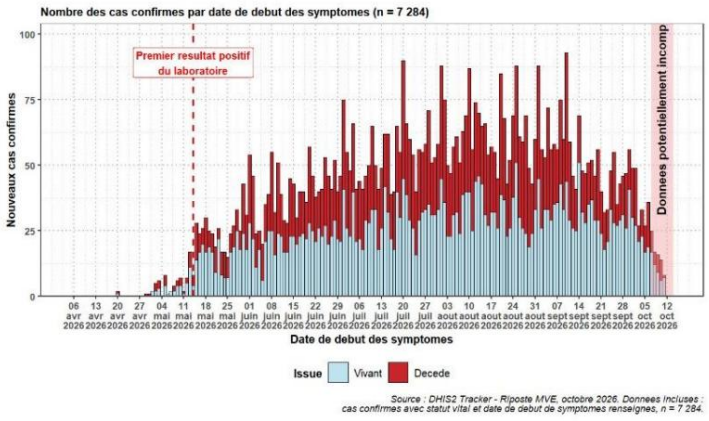
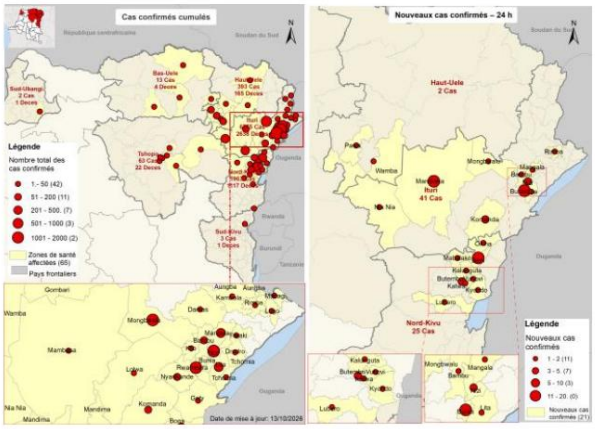


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1.COORDINATION

1.1. Main Actions

• Support for response activities in the affected provinces through the technical pillars.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

As of October 13, 2026, 3,140 alerts had been reported in the six provinces that submitted their data (Ituri, North Kivu, Haut-Uélé, Bas-Uélé, Tshopo, and Sud-Ubangi). Among them, 3,080
(93.8%) were verified, of which 513 (16.7%) were validated as suspected cases. Of these 513 suspected cases, 506 (98.6%) were investigated, of which 317 (62.6%) were transferred to CT/CTE; 7 suspected cases did not

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No investigations were conducted in Ituri, and none of the 10 suspects from Bas-Uélé were transferred. South Kivu did not provide its data.

Of the **32,864** contacts being monitored in the six provinces that submitted their data, 26,870 contacts were seen in the last 24 hours, representing a follow-up rate of **81.8%,** up from the previous day **(77.6%)** but below the required threshold of 95.0%. This rate was 98.3% **(468/476) in Bas-Uélé, 95.5% (42/44) in Sud-Ubangi, 92.0% (1,432/1,557) in Haut-Uélé, 84.3% (741/879)** in Tshopo, **83.2% (10,509/12,637)** in Ituri, and **79.2% (13,678/17,271)** in Nord-Kivu, where **1,750** contacts were never seen, including **1,418 in Beni (81.0%).** Only Bas-Uélé and Sud-Ubangi reached the **95% threshold.**

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

On October 13, nine (9) alerts were notified to the PoE/PoC: five (5) in Ituri, including a lifeless body from Arua (Uganda) to the PoC Sans Complainte (AS Malaba, ZS of Ariwara), validated and sampled, one alert validated at DGRPI Nizi whose subject refused his transfer to the CTE and three alerts invalidated after secondary screening (Biakato Mayi and Iga Barrière); two (2) in North Kivu, at the PoCs of Mukulia (ZS of Beni) and Foner Kasindi, validated and referred to the CT of Mukulia and the CS of Kasindi; and two (2) in South Kivu, two lifeless bodies repatriated from Burundi intercepted at the PoE of Kavimvira (ZS of Uvira), validated without swab.

Table 4. Status of key PoE/PoC indicators by province, as of October 13, 2026

Indicators	Ituri	North Kivu	South Kivu	Boss	Haut-Uélé	Global
Report completeness	98,4% (60/61)	90,5% (19/21)	55,6% (5/9)	100,0% (18/18)	90,0% (9/10)	93,3% (111/119)
% of functional strategic PoE/PoC	100%	100%	ND	100%	90%	ND
Number of people who have switched to PoE/PoC	152 144	92 627	8 187	18 152	17 928	289 038
% of travelers screened	98,1%	98,1%	98,4%	98,4%	99,1%	98,2%
% of travelers who washed their hands	97,8%	98,1%	98,4%	98,1%	99,0%	98,0%
% of travelers made aware	99,6%	98,1%	100,0%	100,0%	100,0%	99,2%
Number of alerts notified	5	2	2	0	0	9
Number of live alerts validated (suspected cases)	1	2	0	0	0	3
Number of bodies recovered today	1	0	2	0	0	3
% of suspected cases transferred to the CTE/CT	0,0%	100,0%	THAT	THAT	THAT	66,7%
% of lifeless bodies swabed/secured	100,0%	THAT	0,0%	THAT	THAT	33,3%
% of suspected cases investigated within 4 h	100,0%	100,0%	THAT	THAT	THAT	100,0%

2.2.2. Challenges

In Ituri, the low encoding of PoE/PoC alerts in the DHIS2 Tracker, the insufficient mobility for supervision in the health zones of Rwampara, Nizi and Bunia, the delay in supplying inputs to some PoCs and the delay in payment of September salaries persist, and the Avakubi PoC remains non-functional due to insecurity; in North Kivu, the completeness remains at 90.5% (19/21), the Butuhe PoC remains non-functional since the security incident of 26/09/2026, the security threat persists against service providers on the Butembo axis, alerts raised by the PoCs are not handled in a timely manner by the alert cells and the PoCs lack consumables (IPC inputs, PPE, monitoring tools), means of communication and a dedicated ambulance.

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2.3. Laboratory

2.3.1. Main actions

- **Ituri: 41 new positive results** (31 living and 10 deaths) out of 196 samples received and analyzed **(positivity: 20.9%).**
- **North Kivu: 25 new positive results** (15 living and 10 dead) out of 157 samples received and analyzed **(positivity: 15.9%).**
- **Haut-Uélé: 2 new positive results** (2 deaths) **out of 23 samples received and analyzed (positivity: 8.7%).**
- **Tshopo: 3 new samples** received and analyzed **(all live),** all tested negative
- **Bas Uélé: 15 new samples** received and analyzed **(14 live and 1 death),** all confirmed. negatives

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- **Ituri:** 9 out of 49 rings were opened (18.4%), 9 out of 18 households were decontaminated (50.0%), and 5 out of 5 health facilities were decontaminated (100%), without the provision of hygiene kits to households, social economy enterprises, or schools. Of 113 blood and sanitation alerts (including 15 postponements from the previous day), 79 blood and sanitation surveys were carried out (69.9%), and 88 out of 96 bodies were swabbed, with 32 failures due to resistance in Bambu, Nia-Nia, Tchomia, Mangala, and Mandima.

- **North Kivu:** 18 out of 24 rings were opened (75.0%), with a significant gap in Beni; 14 out of 14 ESSs were decontaminated (100%) and 8 out of 19 households (42.1%), with low coverage in Kalunguta and Musienene, as none of the 8 decontaminated households received kits; 10 IPC assessments (scorecard) were conducted with an average score of 36.9%; 375 providers were briefed and 157 ESS follow-ups were carried out; 15,000 liters of water were delivered to the Katwa CTE (CAUB/UNICEF); and 2 PPLs were infected among the 27 confirmed cases on October 12 (7.4%). Of 39 death alerts, 39 DHS were carried out and 39 bodies were swabed.

- **Haut-Uélé:** 2 out of 2 rings were opened (100%), 5 out of 5 households were decontaminated (100%), and 3 out of 3 ESS (100%) were completed in Isiro, Rungu, and Pawa; 3 ESS were assessed (45.0% at Notre-Dame de Neisu Hospital, 29.0% at the CS, and 25.8% at the Bafwabaka Hospital), and 53 PPLs (People Living with Distress) were briefed (10 in Isiro, 20 in Pawa, 18 in Wamba, and 5 in Boma Mangbetu). Of 9 EDS (Emergency Disposal) alerts, 6 EDS were carried out (66.7%), and 7 bodies were swabed without failure. 2 EDS remain in progress in Isiro due to a lack of graves and coffins.

- **Tshopo:** 19 out of 19 SSEs were monitored in 5 health zones and 223 providers were briefed on standard precautions, triage and screening; 5 premises and 2 latrines were decontaminated and 2 mattresses were incinerated at Afya Hospital
  Bora, 6 replacement mattresses provided in Makiso-Kisangani, and a thorough investigation was conducted regarding a patient infected during treatment. The cumulative total reached 53 out of 53 wards open (100%) and 35 out of 38 households decontaminated (92.1%); out of 3 EDS alerts and 4 reports, 3 bodies were swabed.

- Bas-Uélé: 8 out of 8 STIs were monitored in the health zones of Buta (2) and Ganga (6), 34 healthcare providers and 12 hygiene workers were briefed, and triage at the Buta General Referral Hospital was strengthened with WHO support, which also provided the Triangle Health Center with infection prevention and control (IPC) supplies; the household and STI of the Dakwa case remain to be decontaminated (0/1) due to a lack of equipment. 2 bodies were swabed, without a complete blood count (CBC), with 3 CBC gaps (1 in Buta and 2 in Ganga) and 1 infected person in Buta

2.4.2. Challenges

- **In North Kivu,** the gap in ring openings in Beni (18/24; 75.0%), the low household decontamination (8/19; 42.1%), the lack of provision of kits to decontaminated households (0/8), the low average PCI score (36.9%), the high-risk exposure of 27 out of 49 assessed PPLs (including 23 out of 38 in Beni) and the infection of 2 PPLs persist.

- **In Bas-Uélé,** 16 out of 20 priority health facilities (80.0%) do not have functional triage, waste management areas, hygienic latrines, garbage cans and waste management systems are insufficient, and the low reporting of community death alerts and low community engagement limit health monitoring (3 gaps)

2.5. Holistic care

2.5.1. Main actions

- **In Ituri,** 80 new admissions (all suspected cases, including 7 in advanced stages) were recorded, compared to 97 discharges, including 13 recoveries, 68 non-cases, 14 deaths (6 confirmed), and 2 discharges without medical advice (1

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confirmed at the CME and 1 suspected at Aru). Thus, 363 patients (127 confirmed and 236 suspected) are hospitalized, representing an occupancy rate of 34.0% (46.0% for suspected and 22.0% for confirmed), including 30 critical patients; 40 patients were evacuated by ambulance and 341 out of 351 patients (97.2%) received a hot meal.

- **In North Kivu,** 115 new admissions (10 confirmed and 105 suspected) were recorded, compared to 92 discharges, including 14 recoveries, 69 non-cases, 8 deaths (5 confirmed and 3 suspected), and 1 discharge without medical advice in Oicha. Thus, 468 patients (167 confirmed and 301 suspected, including 55 children under 5) are hospitalized: 328 in Ebola Treatment Centers (ETCs) out of 374 beds, representing an occupancy rate of 87.7%; 5 patients are in critical condition (ETC in Beni, CT in Kitatumba, and CT **in Oicha), and all 23 ambulance trips were completed.**

In Haut-Uélé, six new admissions (one confirmed case in Isiro and five suspected cases in Pawa and Wamba) were recorded, compared to two recoveries and four negative cases in Boma Mangbetu, two confirmed deaths (Pawa and Wamba), and one suspected death in Wamba. Currently, 45 patients (17 confirmed and 28 suspected) are hospitalized out of 169 beds, representing an occupancy rate of 26.6%. One confirmed case is in critical condition in Pawa, and two suspected cases are in critical condition (Pawa and Wamba). Forty-three patients, 52 caregivers, and 70 people with disabilities received hot meals, and two patients were transported by ambulance. The province's total number of recoveries has reached 157.

- **In Tshopo,** four new admissions (all suspected cases, including two in Bafwasende) were recorded, compared to one discharge—the recovery of a medical student. Thus, 16 patients (6 confirmed and 10 suspected) are hospitalized out of 36 beds, representing an occupancy rate of 44.4%, with no patients in serious condition. Two confirmed cases from Bafwasende have escaped, and the 54th confirmed case (a motorcyclist) remains unaccounted for. Since the beginning of the outbreak, 461 people have received the experimental treatment, including 41 confirmed cases, and 420 have received post-exposure prophylaxis.

- In Bas-Uélé, no new confirmed cases have been admitted; 11 suspects remain isolated (Viadana 3, Bondo 3, Ganga 2, Dakwa 1, Likati 1 and Titule 1), without discharge, recovery or death, and 18 verified patients remain in the community due to a lack of adequate isolation conditions; the monitoring of the confirmed child isolated in Dingila and the confirmed case in Dakwa continues.

2.5.2. Challenges

In Ituri, the low capacity of some CTE/CT (Mambasa, Kilo, Lolwa, Mangala and Biakato), the care of 30 critical patients, the lack of food for 10 patients in the Mandima health zone (CT Mandima and CT Makeke) and the low reporting of ambulance movements (21/33) remain worrying.

In North Kivu, the saturation of suspected beds (164/165; 99.4%), the care of 140 patients (29.9%) outside of CTE/CT facilities, an ambulance response capacity limited to 50.0% (14/28) and the incompleteness of ambulance data (Kalunguta) remain concerning.
In Haut-Uélé, poor network coverage in some health zones, demotivation among some staff, insufficient nutritional inputs, and inadequate water and electricity supplies to some treatment centers remain challenges to be addressed.

In Tshopo, the lack of resuscitation equipment at the CTE and of staff for post-exposure prophylaxis weakens the care provided.

In Bas-Uélé, the lack of appropriate nutritional care for isolated patients, of ambulances for the transfer of patients and of standardized CTE, still under construction at the HGR of Buta, remains a major operational challenge.

2.6. Vaccination

2.6.1. Main actions

- **In Ituri,** the vaccination of PPLs with the Ervebo vaccine continued: 69 people were vaccinated on October 13, bringing the cumulative total to 1,007 vaccinated, and research on the effectiveness of the Ervebo vaccine against the BDBV strain continued.

2.6.2. Challenges

- **In Ituri,** the target for PPL and TPL remains to be redefined for the calculation of vaccination coverage (1,007 vaccinated) for a provisional target of 1,000).
- In Bas-Uélé, the provincial stock of Ervebo vaccine is out of stock (0 doses), the start-up of the UCCs is not effective and the funds for vaccination are still not available; vaccination coverage remains at 11.4% (1,500/13,202).

- **In North Kivu,** indicators of continuity of essential services were analyzed in the health zones of Butembo, Beni, Katwa and Musienene for the purpose of supervision.
- **In Bas-Uélé,** the Dakwa General Referral Hospital is partially operational, with 13 staff members present out of 36 planned (36.1%), a decrease in attendance and the absence of a water point and a waste area.

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2.7. Continuity of Care

2.7.1. Main actions

- **In Tshopo,** the assessment by the Peace Committee showed an effective triage area and a medical director trained on Ebola, but a shortage of HIV medication, the absence of a functional incinerator (use of a burner), a laboratory technician not trained in malaria diagnosis and only 3 agents vaccinated against Ebola out of 16 (18.8%); the recovery plan for REGIDESO was developed and the monitoring of PPL at the CTM and CTE as well as that of recoveries with comorbidities at the Makiso-Kisangani General Referral Hospital continued.

- **In** North Kivu, the baseline assessment of the Mambangu Garrison Military Hospital revealed the absence of triage, PPE, an incinerator, handwashing facilities and vital medical equipment, an unsuitable maternity ward and a poorly organized antenatal care service for a camp of 4,000 households; a recovery plan was produced with the General Referral Hospital team, an urgent order of PPE was placed to supply the facility and the schedules were reorganized

- **In Bas-Uélé,** the evaluation of the Dakwa General Referral Hospital (July to September 2026) showed 21 staff members present out of a planned 50 (42.0%), insufficient stocks of essential medicines and supplies, limited availability of PPE, and a lack of support from partners for the facility

2.8. Risk Communication and Community Engagement (RCCE)

2.8.1. Main actions

- **In Ituri,** 21,571 home visits reached 72,373 people; 659 educational talks brought together 5,093 people, 232 mass awareness campaigns reached 12,917 people, 4 community debates reached 277 people and 9 advocacy sessions reached 100 authorities and leaders; 8 spots were broadcast on 36 radio stations and 2 programs were organized.
  In total, 340 community alerts, including 159 deaths, were reported; 69 out of 87 feedback requests (79.3%) and 4 out of 8 incidents (50.0%) were resolved within 48 hours. The CREC facilitated the pre-listing of 163 contacts around 5 confirmed cases, the decontamination of 14 out of 14 households (100%), 31 out of 38 EDS (81.6%), the announcement of 3 out of 3 positive results, and the transfer of 20 out of 20 suspected cases to the CTE. 59 RECOs from the Makabo Health District (Rwampara) were briefed on SBC. The completeness of CREC reports is only 35.7% (10/28 health zones).

- **In Bas-Uélé,** 234 home visits reached 632 people (AS Agri Uele, Bambili, Sopi, Dembia and Bambesa), 1 educational talk reached 9 people at the Ganga General Referral Hospital and 1 mass awareness campaign reached 234 students at the La Joie school complex (Bambili); the 7 community alerts notified and the feedback received were all dealt with and the CREC supported the collection of two bodies and the EDS at Mission village (Ganga).

- **In North Kivu,** 14,718 home visits reached 53,378 people, 259 educational talks reached 5,895 people, 52 mass communications reached 6,742 people and 7 briefings reached 101 people; 469 community alerts were notified out of 650 expected (72.2%), 13 feedbacks out of 19 (68.4%) and 1 incident out of 8 (12.5%) were resolved within 48 hours and 1,867 people out of 4,750 (39.3%) were engaged in community actions; community dialogues mobilized 30 leaders from AS Kimbulu (Musienene) and 36 private healthcare workers from AS Mondo (Butembo) and CREC supported the reintegration of a healed woman in Mabalako.

2.9. Mental Health and Psychosocial Support (MHPSS)

2.9.1. Main actions

- **In Ituri,** 23 out of 40 confirmed cases (57.5%), 64 out of 108 confirmed cases (59.3%), 32 out of 80 new suspected cases (40.0%), and 75 out of 156 suspected cases (48.1%) received SMSPS interventions. The teams supported 320 caregivers and 50 people living with HIV, monitored 26 out of 29 recoveries (89.7%) in the community and 15 out of 15 children in daycare, provided support to 14 out of 19 separated and orphaned households, reintegrated 7 out of 13 recoveries, and reported 44 results, 23 of which were positive; 77 children are currently being monitored.

- **In North Kivu,** 23 of the last 27 confirmed cases (85.2%), 312 confirmed cases under follow-up, 148 new suspected cases and 276 suspected cases under follow-up benefited from SMSPS interventions; the teams accompanied 98 households and 32 households of separated and orphaned children out of 50 (64.0%), supported 69 PPLs, followed up with 23 children in the community and 5 out of 5 children in daycare, reintegrated 8 recovered individuals, followed up with 9 out of 12 recovered individuals (75.0%) in the community and announced 101 results, including 27 positive ones; 384 people benefited from psychoeducation with the support of UNICE.

- **In Tshopo,** 5 out of 5 confirmed cases under follow-up (100%), 2 out of 2 new suspects and 5 out of 5 suspects under follow-up benefited from SMSPS interventions in 5 out of 7 health zones that notified (71.4%); 232 people benefited from psychoeducation, 8 PPLs were supported, 4 results, including 3 positive ones, were announced and 1 out of 1 resistance was broken.

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2.9.2. Challenges

- **In Ituri,** only 14 out of 28 affected health zones are covered (50.0%), the Mandima health zone, which concentrates 32% of the cases notified today, is not covered and inter-pillar collaboration needs to be strengthened for interventions around cases in Bunia and Mambasa.

- **In Bas-Uélé,** SMSPS coverage remains embryonic, with activities having only just begun in the ZS from Viadana.

- **In Haut-Uélé,** the mobility of community health workers remains difficult in Wamba and Boma Mangbetu, nutritional kits are lacking for weaned infants in Wamba and Isiro, the community health worker lacks computer equipment, and data collection remains weak in Boma Mangbetu and Pawa

- In Tshopo, there is a need to train the APS (Auxiliary Police Officers) of the Wanie-Rukula Health Zone and an insufficient number of units communication and motorcycles for the movement of the pillar.

2.10. Logistics

2.10.1. Main actions.

Ensuring the mobility of all field teams in the affected provinces; in Bas-Uélé, the construction of the CTE at the HGR in Buta (partitioning of rooms and toilets) and the rehabilitation of the DPS offices with the support of the WHO are continuing, the surveillance teams have been deployed to Ganga and Viadana and the packaging of inputs for the remaining ZS is planned; in North Kivu, the CTE in Katwa has been supplied with water (15,000 liters) and the opening of the CTE in Musienene remains planned for October 15.

2.11. Security

2.12. Main actions

In North Kivu, the EDS team was escorted to the burial of 3 confirmed cases at the Bundji cemetery, and a provisional map of accommodation sites and safe neighborhoods was established in the Beni health zone (municipalities of Bungulu, Mulekera, Beu, and Rwenzori); in Bas-Uélé, the insufficient support from security services for PoE/PoC activities was analyzed with the PoE/PoC pillar.

2.13. PSEA (Prevention of Sexual Exploitation and Abuse)

2.13.1. Main actions

- **In Ituri,** 21 humanitarian actors, service providers, and government members (including 13 women) were trained on the PSEAH in Bunia; 87 community relays, village chiefs, and leaders (36 women and 51 men) from the Mandima and Mayuwano health districts began a two-day briefing on the village-based approach integrating the PSEAH and accountability; and 445 teachers and students from the Odhama Yoko Primary School (Aru health district) were sensitized. The posting of messages and the distribution of leaflets continued in the health zones of Bunia, Aru, Mongbwalu, Mambasa, and Fataki. In Tshopo, 85 actors from six pillars (52 women and 33 men) were briefed on the PSEAH with WHO support and signed the code of conduct; and 641 people were sensitized in Mangobo, Kabondo, and Makiso-Kisangani. In Haut-Uélé, 152 students from ISTM Isiro, 6 agents from PNMLS Isiro and 15 members of the EDS team from Boma Mangbetu were sensitized on the PSEA, the referral mechanism and the toll-free numbers 495555 and 151, the PNMLS agents and the EDS team having signed the code of good conduct.

- **In North Kivu,** 1,281 people were sensitized on the PSEA and the toll-free numbers 151 and 495555 during community sessions in the health zones of Katwa (1,187 beneficiaries in the AS Vungi, Masulu and Rughenda), Butembo (63 people in the AS Makasi and Matanda) and Beni (31 people in the AS Mabolio), and 55 participants (39 men and 16 women) were briefed on the PSEA in Katwa with the support of the IMC.

2.13.2. Challenges

At the national level, only 2 out of 7 health districts (28.6%), Ituri and North Kivu, reported their PSE (Public Health Support and Education) activities to the national coordination, with only one supporting partner reporting (WHO). Coverage of affected areas remains low, visibility tools are insufficient, some partners are not aligned, and the effectiveness of Community Health Centers (CHCs) is low. The proportion of health zones aware of the reporting mechanism remains low in Ituri (36.1%; 13/36) and North Kivu (17.6%; 6/34). In Tshopo, 7 out of 23 health zones are covered, and 85 out of 270 staff members have been trained (31.5%).

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MESSAGES CLES

ÿ In the last 24 hours, 506 suspected cases out of 513 (98.6%) were investigated in the provinces that submitted their data. However, 26 suspected cases remain uninvestigated in Ituri. Furthermore, the contact tracing rate decreased from 77.6% the previous day to 81.8%, although it remains above the required threshold of 95.0%. This situation necessitates strengthening epidemiological surveillance and contact tracing to limit the risk of community transmission.

Maintain a level of vigilance regarding epidemiological surveillance aspects, consolidate IPC and CREC actions at the community level while providing quality care in dedicated care structures with adequate reception capacities.

TIMELINE OF KEY EVENTS

ÿ From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.

ÿ Between May 17 and 18, 2026, national authorities , with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.

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